

Report to: Att: Chris Betro Imedical C/O Imedical 1 Union St PYRMONT 2009	Name: SCHLEGER, SHANE Addr: C/O IMEDICAL 1 UNION ST PYRMONT 2009 Phone: 0415994541 Your Ref: 386678 *** URGENT ***	D.O.B.: 01/01/85 Sex: M Lab Ref: 26-16463126 Collected: 22/06/26 Time Coll: 11:53 Reported: 25/06/26
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TESTS: Final report. Thank you for your referral.

CLINICAL NOTES:

Serum Lipoprotein(a)	< 19 nmol/L
(previous units)	< 80 mg/L

Roche method in use.

Population screening for elevated levels of Lp(a) is not currently recommended.

Estimated atherosclerotic cardiovascular disease risk*	Lp(a) level (nmol/L)	Percentile of population
Low	<100	<80th
Moderate	100-200	80-95th
High	200-400	95-99th
Very high	>400	>99th

* Australian Absolute Risk Score where low is <10% risk, moderate is a 10-15% risk and high is a >15% risk of CVD within the next 5 years.

Source: Australian Atherosclerosis Society. Position Statement on Lipoprotein(a): Clinical and Implementation Recommendations. Heart, Lung and Circulation. 2022

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Aldosterone	250	pmol/L	Upright	(100-950)
Renin	20	mU/L	Upright	(3-40)
Aldosterone/Renin Ratio	13			(2-75)

Ranges assume normal salt intake and Potassium levels, and not on antihypertensives or diuretics.

Normal values, not suggestive of HyperAldosterone syndromes.

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SERUM HIGH SENSITIVITY C-REACTIVE PROTEIN (CRP)

Request Number	16463126
Date Collected	22 Jun 26
Time Collected	11:53
hsCRP (< 4.00) mg/L	0.73

The CDC / AHA recommend the following hsCRP cut-off points (tertiles) for cardiovascular disease risk assessment:

hsCRP level (mg/L)	Relative Risk
<1.0	Low
1.0 - 3.0	Average
>3.0	High

The average of two CRP tests, ideally taken two weeks apart, produces a more stable estimate of this marker.

A CRP greater than 9.5mg/L should prompt a search for a source of infection or inflammation.

PLEASE NOTE: From 12/03/2026, High Sensitive CRP (hs-CRP) testing will be performed on the Siemens Atellica platform.

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CLINICAL NOTES:

SERUM CHEMISTRY

Request Number	67505589	16463126
Date Collected	27 Jan 26	22 Jun 26
Time Collected	08:33	11:53
Specimen Type: Serum		

Haemolysis	Nil	Nil
Icterus	Nil	Nil
Lipaemia	Nil	Nil

Na	(135-145)	mmol/L	138	140
K	(3.5-5.2)	mmol/L	3.9	4.6
Cl	(95-110)	mmol/L	103	101
HCO3	(22-32)	mmol/L	27	28
An Gap	(7-17)	mmol/L	12	16
Urea	(3.0-8.5)	mmol/L	7.0	5.8
Creat	(60-110)	umol/L	107	98
eGFR	mL/min/1.73sqM		74	82
Urate	(0.20-0.42)	mmol/L	0.42	0.53
Bili	(< 24)	umol/L	19	17
AST	(< 36)	U/L	23	45
ALT	(< 41)	U/L	31	34
GGT	(< 51)	U/L	29	15
Alk Phos	(40-120)	U/L	61	60
Protein	(60-80)	g/L	73	75
Albumin	(38-50)	g/L	46	46
Glob	(20-32)	g/L	27	29
Ca	(2.10-2.60)	mmol/L	2.41	2.50
Corr Ca	(2.10-2.60)	mmol/L	2.35	2.44
PO4	(0.75-1.50)	mmol/L	0.84	0.90
Mg	(0.70-1.10)	mmol/L		0.83

eGFR values between 60 and 89 mL/min/1.73m² should be interpreted with caution. These results are only consistent with CKD in the presence of other evidence such as microalbuminuria, proteinuria or haematuria.
 Ref: Lamb EJ et al in Ann Clin Biochem 2005; 42:321-345.

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SERUM/PLASMA GLUCOSE

Request Number	67505589	16463126
Date Collected	27 Jan 26	22 Jun 26
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Fasting status	Fasting	Fasting
Serum (3.4-5.4) mmol/L	4.8	5.1

Normal glucose concentration.

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CLINICAL NOTES:

IRON STUDIES

Specimen Type: Serum			
Serum Iron	31	umol/L	(10-30)
Transferrin	27	umol/L	(27-46)
Transferrin Saturation	59	%	(13-45)
Serum Ferritin	90	ug/L	(30-300)

Raised transferrin saturation may be the earliest sign of iron overload.
 Suggest repeat fasting iron studies to confirm the finding.

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CLINICAL NOTES:

THYROID PROFILE

Request Number	67505589	16463126
Date Collected	27 Jan 26	22 Jun 26
Time Collected	08:33	11:53
Specimen Type:	Serum	
TSH	(0.5-4.0) mIU/L	1.6 1.3
FT4	(10-20) pmol/L	18 17
FT3	(3.5-6.5) pmol/L	6.5 6.3

Result(s) consistent with euthyroidism.

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SERUM HORMONE PROFILE

Specimen Type: Serum							
Request	Date	FSH	LH	PROG	E2 (ATEL)	E2 (BECK)	LH/FSH
Number	Collected	IU/L	IU/L	nmol/L	pmol/L	pmol/L	Ratio
67505589	27 Jan 26	< 1	< 0.1	2	191		
16463126	22 Jun 26	1	0.1	2	207		
Reference Ranges		FSH	LH	PROG	E2 (ATEL)		
Males < 60 yrs		<7	<7	0.9-3.9	<150		
> 60 yrs		<14	<11	0.9-3.9	<150		

PLEASE NOTE:

- 'E2 (ATEL)' - Oestradiol by Siemens Atellica assay
- 'E2 (BECK)' - Oestradiol by Beckman Access assay

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PROLACTIN

Specimen Type: Serum

Request Number	Date Collected Ref Range	Prolactin mIU/L (40-450)
67505589	27/01/26	260
16463126	22/06/26	169

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PROSTATE SPECIFIC ANTIGEN

Total PSA (Alinity) 1.2 ug/L

Reference Interval Total PSA ug/L: (0.25 - 2.20)

For men under 70, if PSA levels are > 3.0 ug/L but < 10 ug/L (or >2 ug/L but < 10 ug/L with a significant family history), confirmatory testing within 1 to 3 months is recommended. Men aged 70 and above, with PSA levels > 5.5 ug/L but < 10 ug/L, should also undergo confirmatory testing within 1 to 3 months. Please note that PSA testing is only recommended for asymptomatic men over 70 if life expectancy is greater than 7 years.

Please note change in PSA reference intervals on 20/11/2023.

It is important that the same method is used for serial testing because PSA values may differ between methods for different patients.

If interpretive assistance is required please contact a pathologist on (02) 9005 7373.

Guidelines regarding PSA testing available at: <https://bit.ly/3SbDaY7>

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CLINICAL NOTES:

LIPID STUDIES

Request Number	67505589	16463126
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Specimen Type:	Serum	

Reference intervals are included for reference only, and interpretation / treatment goals should be guided by patient-specific cardiovascular risk assessment (see Australian Cardiovascular Risk Charts. Alternatively, the web-site www.cvdcheck.org.au can be accessed in order to complete a risk assessment for individual patients.)

Haemolysis	Nil	Nil
Icterus	Nil	Nil
Lipaemia	Nil	Nil

Fasting status			Fasting	Fasting
Chol	(3.9-5.2)	mmol/L	4.4	3.9
Trig	(0.5-1.7)	mmol/L	1.0	0.8
HDL	(1.0-2.0)	mmol/L		1.1
LDL	(1.5-3.4)	mmol/L		2.4
Non-HDL	(< 3.4)	mmol/L		2.8
Chol/HDL	(< 5.0)			3.5

NVDPA TARGET LIPID RANGES (MMOL/L) FOR PATIENTS AT HIGH / MODERATE RISK OF CARDIOVASCULAR DISEASE:

TOTAL CHOLESTEROL	<4.0
TRIGS (FASTING)	<2.0
HDL-C	>= 1.0
LDL-C	<2.0
NON HDL-C	<2.5

LDL-C exceeds target for higher risk patients and may be excessive in some individuals.

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CLINICAL NOTES:

HAEMATOLOGY

Request Number	67505589	16463126
Date Collected	27 Jan 26	22 Jun 26
Time Collected	08:33	11:53
Specimen Type: EDTA		
Hb (130-180) g/L	169	177
Hct (0.40-0.54)	0.50	0.51
RCC (4.5-6.5) $\times 10^{12}/L$	5.6	5.9
MCV (79-99) fL	89	87
MCH (27-34) pg	30	30
MCHC (320-360) g/L	338	344
RDW (10.0-17.0) %	13.3	12.4
WBC (4.0-11.0) $\times 10^9/L$	7.7	6.3
Neut (2.0-7.5) $\times 10^9/L$	3.7	3.7
Lymph (1.0-4.0) $\times 10^9/L$	3.1	2.1
Mono (0.2-1.0) $\times 10^9/L$	0.6	0.4
Eos (< 0.7) $\times 10^9/L$	0.2	0.1
Baso (< 0.2) $\times 10^9/L$	0.1	0.1
Plat (150-400) $\times 10^9/L$	286	305

HAEMATOLOGY: FBC parameters are within reference range.

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CLINICAL NOTES:

INSULIN-LIKE GROWTH FACTOR-1 (IGF-1)

Specimen Type: Serum

IGF-1 (Somatomedin C) 21 nmol/L (13-35)

Insulin-like growth factor 1 (IGF1) and insulin-like growth factor-binding protein 3 (IGFBP3) measurements can be used to assess growth hormone (GH) excess or deficiency. For all applications, IGF1 measurement has generally been shown to have superior diagnostic accuracy. In particular, for the diagnosis and follow-up of acromegaly and gigantism, IGFBP3 measurement adds little extra diagnostic information.

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SERUM CORTISOL

Time	11:53
Cortisol	398 nmol/L

AM Reference Interval - 145 - 619 nmol/L

PM Reference Interval - 95 - 462 nmol/L

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GLYCATED HAEMOGLOBIN (HbA1c)

Specimen Type: EDTA

HbA1c- NGSP	4.5	%	(4.0-6.0)
HbA1c- IFCC	26	mmol/mol	(20-42)

The WHO recommends that an HbA1c cut-off of $\geq 6.5\%$ (48 mmol/mol) is used to diagnose type 2 diabetes.

While it is recognised that HbA1c levels approaching this cut-off place patients at increasingly higher risk of developing diabetes ($<6.5\%$), there is no consensus as to exactly which cut-off at the lower end of the continuum to use for categorising patients as high risk. Various groups quote lower limits for at-risk patients that vary between 5.5% and 6.0% (37 and 42 mmol/mol).

Please note that HbA1c should not be used for diagnosing diabetes mellitus in the following circumstances:

- Children and young people
- Pregnancy - current or within the past 2 months
- Suspected Type 1 diabetes mellitus
- Symptoms of diabetes for <2 months
- Patients who are acutely ill
- Patients taking drugs that can cause rapid onset hyperglycaemia such as corticosteroids, antipsychotic drugs
- Acute pancreatic damage or pancreatic surgery
- Kidney failure
- Patients being treated for HIV infection

Please be cautious when requesting or interpreting HbA1c when patients:

- May have an abnormal haemoglobin
- May be anaemic
- May have an altered red cell lifespan (e.g. post-splenectomy)
- May have had a recent blood transfusion

The Healius Pathology Network will bring a dedicated CPD opportunity to clinicians in early 2026 to detect and manage pre-diabetes and diabetes. To pre-register for the program and access the Diabetes Watch report click here: <https://www.healius.com.au/diabetes>

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SERUM ANDROGENS

Total Testosterone (Siemens)	40.1	nmol/L	(8.3-29)
Sex Hormone Binding Globulin	28	nmol/L	(11-71)
DHEAS	6.6	umol/L	(0.9-10)
Calculated Free Testosterone	1034.5	pmol/L	(255.0-725.0)

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SERUM APOLIPOPROTEIN STUDIES

Apolipoprotein A1	1.2 g/L
Apolipoprotein B	0.7 g/L
Apolipoprotein B/A1 ratio	0.6

Guidelines from EAS and EFLM for apolipoproteins:

Parameter Increased CVD risk:

ApoB ≥ 1 g/L

ApoA-1 Men ≤ 1.2 g/L

Women ≤ 1.4 g/L

ApoB level < 0.1 g/L: genetic abetalipoproteinaemia

ApoA-1 level < 0.1 g/L: genetic hypoalphalipoproteinaemia

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SERUM GROWTH HORMONE

Growth Hormone < 1.0 mIU/L (< 3.1)

In children, Growth Hormone should be > 20 mIU/L following any appropriate stimulus (e.g. sleep, exercise, hypoglycaemia, arginine). Random values are usually low. Growth Hormone is not recommended to diagnose Growth Hormone excess or deficiency. IGF-1 is the preferred marker, especially for the investigation of Growth Hormone excess.